Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

OPERATIONAL POLICIES

YG OPG 003 V 1.0 DECEMBER 2017

Management of Critically Ill Children in a DGH Critical Care Unit

REVIEW BY DECEMBER 2020

Author(s): Dr Ed Farley-Hills
Reviewer: Critical Care Consultants

Key Points

- This policy provides guidance on the management of Level 3 children in YG in the event of there being no availability of tertiary intensive care beds.

Situation

In the event of there not being any tertiary paediatric intensive care beds, all adult ITUs in North Wales may need to ventilate a child for a prolonged period of time.

Background

The Welsh Government has mandated that all acute hospitals in Wales must have an area to safely ventilate a child for a prolonged period of time (3-4 days). PICU tertiary centres are coming under intense pressure for beds especially with winter approaching. There may be times where they are unable to accommodate a paediatric patient, in which case they will need to be looked after in the DGH. The PICUs will prioritise the younger/smaller/more complicated children which will leave the DGHs to potentially look after the older child.

Assessment

There are 2 potential areas in YG.

1) Dewi ward resus area.

Advantages-Familiar area, paediatric nurses and doctors nearby. Paediatric equipment to hand.
Disadvantages- Anaesthetic presence difficult to maintain for prolonged periods due to distance. Ie they have other duties too.

2) Cybi ward

Advantages-familiar, able to maintain anaesthetic presence easily
Disadvantages- No readily available presence of paediatric doctors or nurses. Cybi nurses not paeds trained. No paeds kit immediately available.

Recommendations

There are advantages and disadvantages to both areas. For the smaller child, Dewi may be better, but for the larger child who has a prolonged stay, Cybi may be more appropriate.

Children Ventilated on Cybi:

Each admission must be risk assessed and the patient will be the joint responsibility of the paediatrician **and** the intensivist/on call consultant anaesthetist.

A clear joint clinical plan must be written and agreed by both paediatric and anaesthetic consultant. There must be regular input from the paediatric consultant on-call and paediatric nurses. The amount of input and physical presence will vary depending on the severity and the age of the child. However there must be a minimum of 24/7 availability of phone advice from both paediatric nurse and paediatric consultant if required. Minimum input from the paediatric consultant will be 2 clinical reviews per day. There is a resident middle grade paediatric doctor available for more immediate needs.

A **risk assessment** will be carried out by the paediatric consultant, anaesthetic consultant, band 7 ITU nurse and band 7 paediatric nurse to decide on where to stabilise and care for the patient based on:

a) Medical skill mix

b) Nursing skill mix

c) Severity of illness.

d) Age of child.

- Equipment must be sourced, appropriate for age. A checklist for age/weight appropriate equipment will be used.
- All drug doses and infusions must be calculated for age and weight. Advised to use- Crashcall Emergency Drug Calculator
- Cybi nurses will endeavour to go on study days to Alder Hey hospital
- Current Paediatric Clinical Guidelines may be viewed at: **http://www.nwts.nhs.uk/clinicalguidelines**

N.B. All Cybi nurses are covered by the RCN to provide emergency care of paediatric patients.